جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : منى فاروق محمد ======

المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ==========

رقم البطاقة : 27207032101921 **رقم التليفون :** 01276333618 **تاريخ تسجيل الإستمارة :** 17-10-2026

العمر(تاريخ الميلاد) : 03-07-1972 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: منى فاروق محمد ======
National ID: 27207032101921

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي
انترناشونال

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 7/12/2025 G

Name	منى فاروق عبد	MRN	3016/2025
		ID	
Age	53	Phone	01112728246
Menopausal status	Pre-menopausal	Marital status	Married.
NRS SCORE	P.S :	Family history	-ve
BMI		REASON OF MDT	New Case
CO morbidity	HTN "hist of Cerebral hge 5y ago" Not Diabetic, Not Cardiac		

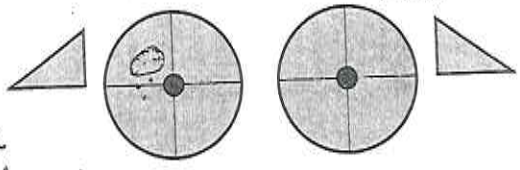
Clinical Data ٢٧٢-٧٠٣٤١٠١٩٤١

Rt breast

BX: Tru Cut : IDC GII

ER:	PR:	HER 2:	Ki67:
7/8	6/8	+1	80%

R) cT2 N2 M0 L
.. Noted mild skin edema ??



Sonomammography: 5/10/2025 ACR BI
(R+) UOQ malignant mass + Small Satellite + ipsilateral axillary LN (L+) Complicated cyst
max 2.3 X1 cm

Mets Workup:
PET-CT: 20/11/2025
- Multicentric Rt breast UOQ neoplasm 2.5×1.8cm SUV 12
- OLQ Rt breast nodule SUV 4.3.
- Multiple Rt axillary + Subpectoral LN. 2.1×2cm SUV 12.7
- No distant deposits.

TNM

Recommendations:
- for Echo, full labs
- MDT to start NAT & type of Surgery

MDT OUTCOME & PLAN
For NACT then RT MRM
Need to CK → HAE → tests X4 → RT MRM

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		Nor Hussein 11/12/2025			

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: مني فاروق محمد محمد	
MR. : 2998	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presents with a history of right breast cancer, classified as T2 N2 multicentric invasive ductal carcinoma, ER 7/8, PR 6/8, HER2-negative. Initial management included a regimen of Ki-67 80, cyclophosphamide 120 mg, and endoxan 1200 mg, with a planned transition to taxotere. Physical examination revealed a 2 × 2 cm mass at the 12 o'clock position of the right breast.

The patient was scheduled for cycle 3 of taxotere at 130 mg. Labs were accepted but the dose was reduced due to an episode of severe respiratory depression during the previous cycle. It was recommended to add solucortef to the premedication regimen, as the patient had missed a dose of methabiogin the day before.

On the same day, for cycle 3 of taxotere 130 mg, labs were pending and the infusion rate was set at 1.5 hours. Solucortif was added to the premedications, and blood pressure monitoring was advised. Ondalenz and methabiogin were also added to the premedication protocol.

During the subsequent visit, the patient had no palpable breast masses. The plan was to proceed with cycle 4 of taxotere at 150 mg, with a focus on the breast and axilla for surgical consultation and to arrange for blood transfusion support. The patient has completed 4 cycles of taxotere.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : شريف الشناوى

يعتمد ,

NHTMRI

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومى للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجى

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name:مني فاروق محمد محمد
Request type: مبادره
Hospital no:
Department:صحه مرأه
Referred by:د/نورهان اسامه
Nature of specimen: tru-cut biopsy
Clinical diagnosis: Right breast mass

Age: 53years Sex: female
Sub request type:
Pathology no.: 608B-2025
Date of result:12/10/2025
Date of biopsy:5/10/2025
I.D. Number:27207032101921

Gross pathology :

Multiple grayish white tissue cores up to 1cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- Ki67

Microscopic picture: Section examined revealed mammary core tissue showed malignant growth formed of tumor clusters, nest and tubules of anaplastic ductal cells with moderate anaplasia and moderate tendency to form tubules dispersed among desmoplastic stroma. No ductal carcinoma insitu component could be seen. There is mild lymphocytic infiltrate. There is no evident lymph vascular tumor emboli invasion or perineural infiltration in core tissue examined.

Immunohistochemical Result:

ER: Moderate nuclear staining in 80% of tumor cells (Allred Score 7/8).
PR: Moderate nuclear staining of 60% of tumor cells (Allred Score 6/8).
Her2: Weak incomplete membranous staining of >10% of tumor cells (Score 1+).
Ki67: Positive nuclear staining of 80% of tumor cell

Diagnosis: Right breast mass, tru-cut biopsy:

Invasive ductal carcinoma (NST), grade II .
(ER: 7/8), (PR: 6/8), (Her2: Score1+), (Ki67: 80%)

Signature:
Dr.Nermine .Nabil.M.D
Dr.Mona Mohamed.M.D

Pathology Lab. Manager

Nermine Nabil, M.D

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Women's Health Outreach Program

Name: Mona farouk Mohamed
Date: 05/10/2025

Bilateral Full field Digital Mammography

Clinical summary: 53 years old lady complain right breast lump.

Technique
Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibrogalndular density (ACR B).
- The right breast shows UOQ focal asymmetry and distortion measures about 4.5 x3.3 cm seen 2.8 cm from the nipple
- **Ultrasound demonstrated UOQ irregular hypoechoic mass lesion seen at 12 O'clock seen 2 cm from the nipple, measuring about 2.3x1 cm with mild internal vasculaitery associated with mild increased skin thickness reaching 4.4 mm**
- **Another Smaller rather defined hypoechoic lesion taller than wider seen at 7O'clock parareolar measures about 4.7 x4.5 mm**
- **Few small simple subcenetermteric cysts seen at the lower quadrant**
- **Multiple enlarged pathological axiliar iymph nodes <5, at level I with effaced hila largest measures about 17 x12 mm with cortical thickness reaching 7 mm**

Left breast

- Scattered fibrogalndular density (ACR B).
- No masses , asymmetric densities , distortion or suspicious microcalcifications
- **Ultrasound demonstrated small well defined cystic lesion with echogenic content seen at 1 O'clock measures 6.4 x4.7 mm**
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast:** Right UOQ malignant looking lesion with a small query satellite seen as described with ipsilateral pathological axillary LN. (BI-RADS 5) Biopsy done at same sitting for histopathological correlation and post contrast enhancement .
- **Left breast** left complicated cyst as described BI-RADS 3) for follow up

Best regards,
Dr. Rania Mostafa
Dr. Nourhan Osama .MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

مؤسسة مستشفى سرطان الأطفال-مصر
Children's Cancer Hospital Foundation - Egypt
57357

Nuclear Medicine Dept.
PET/CT Unit

Name:Mona Farouk Mohamed	**Date:20/11/2025**
Referred by: Prof. Ahmed Elbastawesy	**ID:30203167**

F18-FDG PET/CT STUDY

Clinical History:

- A 53-year-old patient has history of recently diagnosed right breast IDC.

Reason for PET/CT study:

- Initial assessment.

Technique:

- Radiopharmaceutical: 296 MBq of 18F FDG.
- Time from 18F-FDG injection to scan: 79 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.
- Reference hepatic activity is of SUVmax 4.8.

COMPARISON: No previous PET CT studies.

PET Scan
[[DOCTYPE:PET_SCAN]]

FINDINGS:

Head and Neck:

- No hyper-metabolic FDG avid cervical LNDs
- Prominent left thyroid lobe with hypodense nodule measures 2cm, for US correlation. No FDG avid thyroid nodules
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Minimal relative decreased metabolic activity over right cerebral lobe with NO hypermetabolic brain lesions, for clinical correlation.

Breasts/loco-regional:

- **A rather defined FDG avid nodular lesions are noted at the UOQ of the right breast, the largest measures about 2.5x1.8cm of SUVmax 12.**
- **Few FDG avid small nodular lesions are noted at the OLQ of the right breast of SUVmax 4.3, likely satellites.**
- **For correlation with US and mammographic studies.**
- **Multiple enlarged hypermetabolic right axillary and subpectoral LNs are noted, the largest measures 2.1x2.2cm of SUVmax 12.7.**
- No left breast FDG avid masses.
- No left axillary lymphadenopathy.

Chest:

- There is no hypermetabolic mediastinal lymphadenopathy.
- NO FDG avid lung nodules or masses detected.
- No metabolically active pericardial or pleural lesions detected.

- Cardiomegaly with physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- There is no hypermetabolic abdominal or pelvic lymphadenopathy.
- Enlarged liver showing homogenous parenchymatous texture with NO FDG avid focal lesions detected.
- The spleen is of average size with NO FDG avid focal lesions detected.
- The pancreas shows normal FDG avidity.
- Gall bladder stones.
- Prominent left suprarenal, no active adrenal nodules.
- Prominent both adnexa showing mild FDG uptake.
- Physiologic FDG excretion is seen in the kidneys and bladder

Musculoskeletal:

- Degenerative changes of the spine.
- Mild FDG uptake related to spinous process of CV5, for follow up.
- Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

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E-mail.info@57357.org

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ص. ب : ا الدواوين - مصر
الهاتف: 19057 (202)
فاكس: 1745 35 25 (202)

PET Scan
[[DOCTYPE:PET_SCAN]]

مؤسسة مستشفى سرطان الأطفال-مصر
Children's Cancer Hospital Foundation - Egypt
57357

Nuclear Medicine Dept.
PET/CT Unit

IMPRESSION:
Metabolically active multicentric right breast neoplasm.
Metabolically active right axillary lymphadenopathy, likely metastatic.
No hypermetabolic definite distant metastatic sites.

Thanks for referral,

Noura Adel, MD
Radiology Consultant

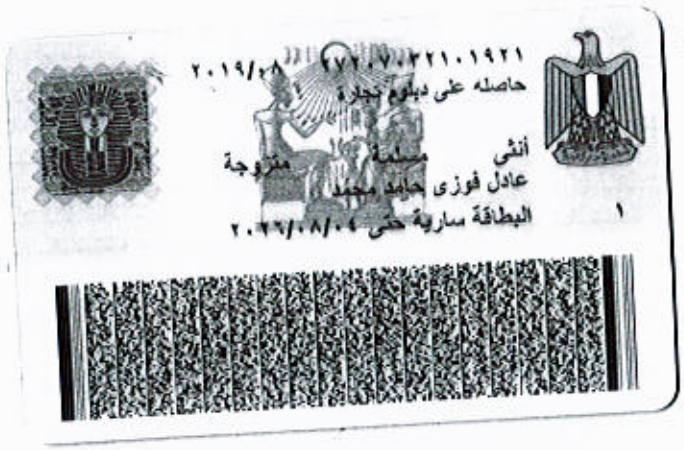
Rehab Maklad, MD
Nuclear Medicine Consultant

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PET Scan
[[DOCTYPE:PET_SCAN]]



DMS

سونارتك واليد



National ID · Front & Back
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